



Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Inpatient, Outpatient and Community Services	Medical, Nursing, Allied Health

Purpose

This standard operational procedure (SOP) outlines best practice guidance for clinicians in recognising and responding to mental state deterioration in all patients, including escalation of care.

This SOP is to be read alongside the overarching **Recognising and Responding to Acute Deterioration (RRAD) Policy**.

Patients and consumers are terms used interchangeably; this SOP will refer to “patients”.

Background

All staff have a role in recognising and responding to mental state deterioration and implementing appropriate risk mitigation strategies to keep patients safe.

A systematic approach to monitoring changes in a patient’s mental state using shared decision making will help to minimise and/or prevent deterioration and adverse events. Deterioration in a patient’s mental state is an adverse outcome that can lead to anxiety, agitation, aggression, self-harm, and suicidality.

Patients experience and express deterioration in their mental state in different ways due to internal factors (including exacerbation of mental illness), psychological distress, physical conditions (including delirium), atypical responses to prescribed treatments (including therapy), or intoxication with licit or illicit substances ^{1,2}.

RPBG has specific referral pathways to ensure patients in inpatient, outpatient and community service settings who experience mental state deterioration, can have access to clinicians and services to assist in their assessment and care. These services include Consultation-Liaison Psychiatrist (C-L), Behaviour Support Team (BeST), Safety After Hours for Everyone (SAFE) and Bentley SAFE (BeSAFE), and Drug and Alcohol.

Links within document

- [Appendix I: Specialist Referral Pathways – General Health](#)
- [Appendix II: RRAD Mental State Escalation Pathway – RPBG General Health Inpatient Service](#)
- [Appendix III: MH Division Assessment Tools](#)
- [Appendix IV: RRAD Mental State Escalation Pathway – MH Division Inpatient Services](#)
- [Appendix V: RRAD Mental State Escalation Pathway – MH Division Community Services](#)
- [Appendix VI: Acute Deterioration in Mental State at Bidi Wungen Kaat Centre \(TCU\)](#)
- [Appendix VII: Acute Deterioration in Mental State at Next Step Inpatient Withdrawal Unit \(IPWU\), Outpatients and Integrated Services Escalation Pathway](#)
- [Appendix VIII: RRAD Mental State Escalation Pathway – RPBG General Health Outpatient Services](#)

RRAD in Mental State, Cognition and Behaviour SOP

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Guiding principles

The guiding principles underlying this SOP are

- Clinicians are alert to the risk of deterioration in mental state, cognition and behaviour for all patients accessing health care, in all healthcare settings
- Clinicians can recognise deterioration, initiate response, and escalate care using appropriate escalation processes. Responding to deterioration in mental state includes comprehensive assessment of potential causes
- Decision making is shared between the multidisciplinary (MDT) team and the patient. Family or personal support persons (PSPs) are involved in accordance with the patient's expressed wishes
- The response to deterioration is person-centred, culturally competent, and recovery-oriented
- The response to deterioration is trauma-informed and should minimise trauma to the patient, their family, PSPs, and clinicians.

Definitions

Mental state	Refers to “a person’s intellectual capacity, emotional state, and general mental health based on clinical observations and interviewing. Mental state comprises mood, behaviour, orientation, judgment, memory, problem-solving ability, and contact with reality.” ²
Mental state deterioration	Refers to “changes in a person’s mental state that indicate the need for closer observation, clinical review, or more frequent review and for the introduction, change or ‘up-scaling’ of therapeutic interventions.” ⁴
Trauma-informed care	“A trauma-informed approach to care acknowledges that health care organisations and care teams need to have a complete picture of a patient’s life situation - past and present - in order to provide effective health care services with a healing orientation.” ⁷

Recognising mental state deterioration

When assessing a patient's mental state, it is crucial to compare their present condition with their unique baseline information. A thorough evaluation must encompass both historical and collateral data on their mental and physiological well-being. Pertinent screenings should be carried out during the presentation, clinical review, and clinical care processes. This initial screening and further re-assessments of behavioural risk should be documented in the *Multidisciplinary Comprehensive Care Plan (MCCP)* +/- Digital Medical Record (DMR) +/- Psychiatric Online Information Service (PSOLIS +/- webPSOLIS) (e.g., service event, *Risk Assessment and Management Plan [RAMP]*).

There are five indicators of deterioration that clinicians need to be aware of and vigilant for:

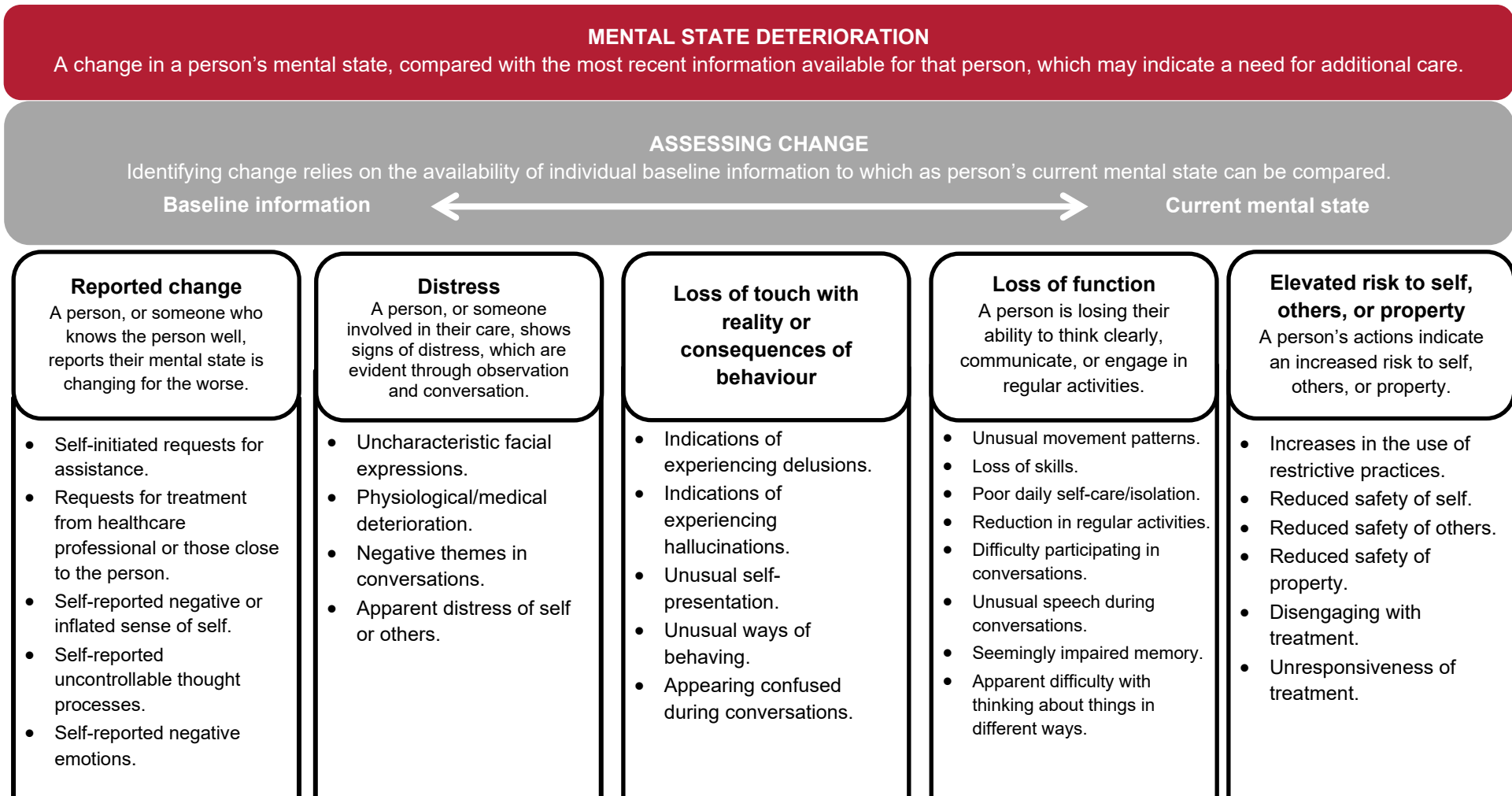
- Reported change by the patient or a person close to them
- Distress
- Loss of touch with reality or consequences of behaviours
- Loss of function
- Elevated risk to self, others, or property.²

Each indicator consists of a cluster of signs of deterioration – refer to [Figure 1: Definition, change, indicators, and clusters of signs of mental state deterioration](#)². Alongside cognitive screening, these indicators can also be used to identify deterioration or change in cognition.

Other indicators of deterioration may include concerns related to:

- Capacity to consent or refuse medical treatment
- Reasons for non-adherence to treatment
- Alcohol and other substance disorders and withdrawal syndrome
- Cerebral disorders resulting in behavioural management problems, aggressive or mental health symptoms e.g., delirium, cognitive impairment
- Abnormal illness behaviour and/or medically unexplained symptoms
- Eating disorders
- Medically unwell patients with known acute or enduring mental illness.

Figure 1: Definition, change, indicators, and clusters of signs of mental state deterioration



Royal Perth Hospital (RPH) – Emergency Department (ED)

All patients presenting at RPH ED will be triaged against the Australian Triage Scale.

Patients presenting with mental health or behavioural concerns will be triaged according to their clinical and situational urgency using the Mental Health Triage Tool ⁵. Where physical and behavioural problems co-exist, the highest appropriate triage category will be applied based on the combined presentation ⁶.

At point of triage, alongside patients self-reporting, the following sources of information may be utilised to assist with the primary assessment:

- Information obtained from paramedics, family, carer, friends, or witnesses
- General practitioner (GP), external services
- WA Police (WAPOL) Handover
- ED Information System (EDIS) Flag
- ED Management Plan - If ED Management plan is in place, this is to be printed off and placed in notes.

During the primary assessment, patients should be asked for any medical history related to mental health issues or known cognitive impairment.

Responding to deterioration

For area specific identification, recognition, referral, and escalation pathways, refer to:

- [Appendix I: Specialist Referral Pathways – General Health](#)
- [Appendix II: RRAD Mental State Escalation Pathway – RPBG General Health Inpatient Service](#)
- [Appendix III: MH Division Assessment Tools](#)
- [Appendix IV: RRAD Mental State Escalation Pathway – MH Division Inpatient Services](#)
- [Appendix V: RRAD Mental State Escalation Pathway – MH Division Community Services](#)
- [Appendix VI: Acute Deterioration in Mental State at Bidi Wungen Kaat Centre \(TCU\)](#)
- [Appendix VII: Acute Deterioration in Mental State at Next Step Inpatient Withdrawal Unit \(IPWU\), Outpatients and Integrated Services Escalation Pathway](#)
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General Health inpatient services and ED

Once a clinician has recognised signs of deterioration, a response is required, where possible, use shared decision making to prevent further deterioration and prevent adverse events. This response should include all relevant members of the interdisciplinary team.

- On identifying concerns, the clinician is to provide initial response including:
 - Identify and escalate any imminent risk concerns
 - Activate appropriate emergency response (Code Black, Code Blue, 000) in a medical or behavioural emergency
 - Reassurance and support
 - Collaborate with the patient to identify current self-management strategies
 - Conduct further screening within scope (or seek specialist advice) – e.g. 4AT, *Alcohol Withdrawal Chart (AWC)*, Nicotine Replacement Therapy (NRT), review physiological observations
 - Identify environmental safety concerns or modifications such as reduced stimulation, removal of potentially dangerous items and equipment, minimisation of ligature points, removal of alcohol-based hand gel
- Inform Shift Coordinator or senior clinician (e.g., BeST, SAFE and BeSAFE, or After Hours Mental Health [MH] Clinical Nurse Specialist [CNS]) of concerns
- Request formal medical assessment by Treating Team – urgency to depend on findings of initial assessment
- Discuss required frequency of visual observations with Shift Coordinator or senior clinician (e.g. BeST, SAFE, BeSAFE, BHS After Hours MH CNS)
- Commence or review required documentation (e.g., *Behaviour Observation Form [BOF]*, patient healthcare record).

In urgent situations (except ED), while awaiting review, the BeST, SAFE, BeSAFE Team, Ward Leader, CNS or BHS After Hours MH CNS can be contacted for advice and support.

Medical assessment by the Treating Team is to include

- Further cognitive screening. (Refer to **Delirium Management CPS**)
- Past and/or current mental health history
- Physiological, reversible causes
- Alcohol and other substance risk of withdrawal assessment (Refer to **Alcohol and Other Drug Intoxication and Withdrawal Management Clinical Guideline**)
- Risk assessment and identification of acute risk.

A documented MDT plan is to be developed and should include consideration of:

- Level of risk and risk mitigation strategies
- Engagement with patient, their carers and/or PSP
- Increase in frequency of observations (visual observations and/or vital signs)
- Non-pharmacological strategies
- Pharmacological options.

General health patients who experience existing mental health issues, or who show signs of mental state deterioration, can be referred to specialist mental health services as per site specific referral pathways (Appendices [I](#) and [II](#)). Ensure documentation of deterioration in mental state includes a risk re-assessment in the *MCCP* and that the *MCCP* aligns with the strategies documented in the MDT plan.

General Health outpatients and community services

The above principles and guidance in recognising deterioration apply for all patients across RPBG. For specific General Health outpatient and community services processes, refer to Appendices [VII](#) and [VIII](#).

Mental Health inpatient and community

The above principles and guidance in recognising deterioration apply for all patients across RPBG. For specific Mental Health Division processes, refer to Appendices [II](#), [III](#), [IV](#), [V](#), [VI](#). All clinicians are to practice within the *Mental Health Act (MHA) 2014* and the [Charter of Mental Health Care Principles \(MHA 2014\)](#).

Escalation to senior management across all RPBG services (i.e., General Health and Mental Health)

When clinical issues or concerns remain unresolved escalation to senior staff (see designations below) and senior management/ Hospital Executive may be required.

The following patient issues must be escalated

- Patients whose behaviour poses a risk to themselves or others that has not responded to local or specialist input
- Patients identified as having sexual safety risks or sexually inappropriate behaviour (refer to **Sexual Safety Policy**)
- Any complex presentation where contradicting views to those of the Treating Team about patients' best interests in ongoing management are expressed by different stakeholders including family, carer, PSP or next of kin (NoK)
- Decisions about patient care that have associated patient, staff, or litigation risk unable to be resolved by Treating Team
- Any critical incident including death, sexual assault, and other assaults, e.g., physical violence resulting in serious injury to the patient, other patients and/or staff
- Where there are delays in admission to inpatient ward resulting in a prolonged time in ED
- Also refer to **Escalation to Consultants Policy**.

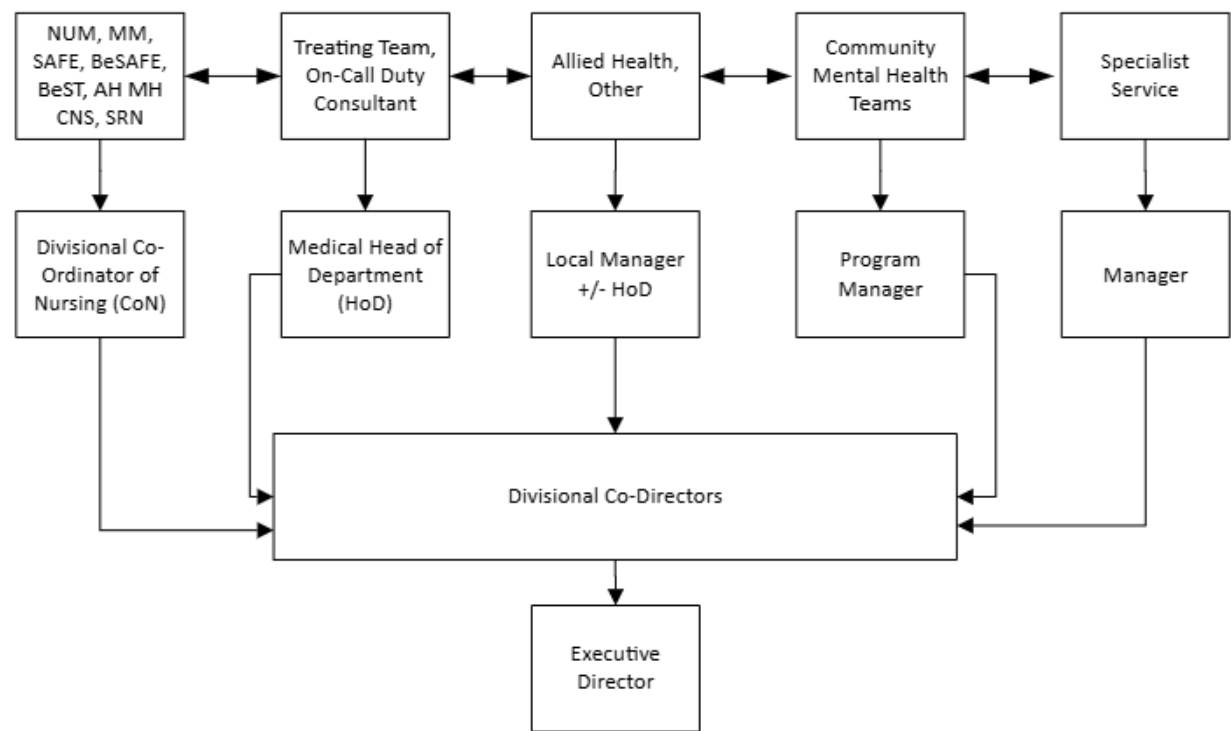
The following Staff designations may escalate issues to senior management (e.g., Hospital Executive, Executive On-Call for after hours or Senior Nurse on Call [SNoC] for nursing issues)

- Ward Nurse Unit Manager (NUM), Midwifery Unit Manager (MUM), Senior Nurse or Shift Coordinator in the absence of the NUM or CNS
- BeST, SAFE or ^{Be}SAFE Team
- BHS After Hours Mental Health CNS
- Treating Consultant or Duty Consultant (on call)
- Local, Service or Program Managers e.g., Allied Health, Transitional Care Unit (TCU) Manager
- Heads of Department (HoDs) or Team Leaders

All stages of escalation and related decisions require staff to clearly document in the DMR and a review date and action plan as a minimum.

Refer to [Table 2: Escalation to senior management pathway](#).

Table 2. Escalation to senior management pathway



Compliance monitoring

Compliance with this document will be monitored by a review of clinical incidents related to RPH and BHS maternity, general health and mental health inpatient, community, or outpatient delay to mental health review.

Facilitator

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Authors / acknowledgements

We acknowledge the following previous site endorsed work and/or contributors used to compile this document.

RRAD in Mental State, Cognition and Behaviour SOP 2021

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Endorsed at RRAD Committee OOS 23/01/2026

Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the RRAD in Mental State, Cognition and Behaviour SOP through consultation with Aboriginal Workforce Engagement Group which includes the EMHS Aboriginal Health Liaison Officers, Aboriginal Health Strategy Team, and Aboriginal Acute Care Coordination Team to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 3014).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Related policies, practice standards, clinical guidelines

Policies and procedures SharePoint

- [Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#)
- [Delirium Management CPS](#)
- [Alcohol and Other Drug Intoxication and Withdrawal Management Clinical Guideline](#)
- [Outpatient Consumer's Clinical Concerns Management SOP](#)
- [Sexual Safety Policy](#)
- [Escalation to Consultants Policy](#)
- [Mental Health Patient, Carer and Visitor Support Post-Critical Incident SOP](#)
- [Prevention and Management of Aggressive Behaviours within MH Inpatient SOP](#)
- [Community Mental Health Services \(CMHS\) Triage to Discharge SOP](#)
- [Emergency Department: Triage and Waiting Room SOP](#)
- [Clinical Handover Policy](#)
- [Comprehensive Care Policy](#)
- [General Health Admission, Transfer and Discharge SOP](#)
- [Prevention and Management of Aggression in Mental Health Inpatient Settings SOP](#)
- [Behaviours of Concern Practice Standard](#)
- [Progressive Risk Assessment SOP](#)
- [Special and Supportive Observations Policy](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Medical Emergency and Cardiorespiratory Arrest Management \(MET\) CPS](#)
- [Potentially Harmful Items Management SOP](#)
- [Ligature Risks Management, Safe Use and Storage of Ligature Cutters Policy](#)
- [Minimum Requirement for Medical Review of Consumers Admitted to Inpatient MH Wards or CMHS Policy](#)

EMHS Policies

- [Care Coordination in Mental Health Policy](#)
- [Notifications requirements under the WA Mental Health Act 2014 Policy](#)
- [Acute Response Mental Health Emergencies/Crisis Policy](#)
- [Mental Health Patient Flow and Bed Capacity Framework](#)

WA Department of Health Policy Hub

- [Recognising and Responding to Acute Deterioration Policy and Guideline](#)
- [Clinical Incident Management \(CIM\) Policy](#)
- [Clinical Handover Policy](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) Guideline](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)

Other

- [Mental Health Act 2014](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [WA Chief Psychiatrist's Standards for Clinical Care 2015](#)
- [Guidelines for Consultation Liaison Psychiatry Services – Chief Psychiatrist of WA](#)

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Related forms

- *Multidisciplinary Comprehensive Care Plan (MCCP)*
- *Behaviour Observation Form (BOF)*
- *Mini Mental State Examination (MMSE)*
- *Adult Observation and Response Chart (AORC)*
- *MH What Matters Most Record (WMMR)*
- *Alcohol Withdrawal Chart (AWC)*
- *Statewide ED WA Police (WAPOL) Handover*
- *AUDIT and DAST-10*
- *Montreal Cognitive Assessment (MoCA) MR398.34*
- *Clozapine Monitoring Form*
- *WA Intramuscular Long-Acting Injection (LAI) Chart*
- *Alcohol, Smoking and Substance Involvement Screening Tool (ASSIST-Lite)*

Related national standards

ACSQHC NSQHS Standard 2nd Edition (2021):

Standard 1 - Clinical Governance,
 Standard 2 - Partnering with consumers
 Standard 5 - Comprehensive Care,
 Standard 6 - Communicating for Safety,
 Standard 8 - Recognising and Responding to Acute Deterioration

References

1. Harris, B., Duggan, M., Batterham, P., Bartlem, K., Clinton-McHarg, T., Dunbar, J.A., Fehily, C., Lawrence, D., Morgan, M., and Rosenbaum, S. (2018). *Australia's Mental and Physical Health Tracker: Background Paper*. Australian Health Policy Collaboration Issues Paper no. 2018-02. Melbourne: AHPC.
2. Gaskin, C. and Dagley, G. (2018). *Recognising Signs of Deterioration in a Person's Mental State*. Sydney: ACSQHC.
3. Australia Commission on Safety and Quality in HealthCare. (2016). *National Consensus Statement: Essential elements for Recognising and Responding to Acute Physiological Deterioration*. Sydney, Australia.
4. Craze L, McGeorge P, Holmes D, Bernardi S, Taylor P, Morris-Yates A, et al. (2014). *Recognising and responding to deterioration in mental state: a scoping review*. ACSQHC. Sydney.
5. Emergency Department Triage Education Kit
<https://www1.health.gov.au/internet/main/publishing.nsf/Content/casemix-ED-Triage+Review+Fact+Sheet+Documents>
6. Australasian College for Emergency Medicine (2016) *Guidelines for the implementation of the Australasian triage Scale in the Emergency Department*.
7. Trauma-Informed Care Implementation Resource Center. *What is Trauma-Informed Care?* <https://www.traumainformedcare.chcs.org/what-is-trauma-informed-care/>
 Accessed February 2026.

Appendix I: Specialist Referral Pathways – General Health

RPHBG have a number of specialist services available to assist with assessment and treatment of patients presenting with an altered mental state.

Emergency Department

Cognitive impairment or older adult

Older adults aged 65 years+ or 45 years + for Aboriginal patients or any person with suspected cognitive impairment can be referred to the **Care Coordination Team (CCT)** for input.

Mental Health Services

Clinical staff can contact the Psychiatry Liaison Nurse (PLN) 24 hours, 7 days a week for specialist input and support.

- **The PLN can be contacted directly or via Switchboard (24 hours, Mon-Sun).**

The ED Psychiatric Registrar undertakes Mental Health reviews and supports the PLN by providing input into determination of patient diagnosis and admission/discharge plan. They can be contacted directly or via switchboard if the PLN is unavailable.

Inpatient wards

Mental Health Services

Clinical staff can contact the following service for specialist advice, assessment, and support prior to referral or during patient admission:

- **Consultation-Liaison (C-L) Psychiatry** Triage Nurse via Switchboard ([RPH Consultant Liaison HealthPoint page](#))
- **After Hours, Psychiatric Registrar directly or via Switchboard (24 hours, Mon – Sun).**

C-L Psychiatry is a Consultant Psychiatrist led MDT that provide assessment, diagnosis and management of patients who experience co-morbid medical and mental health issues:

- Refer to [Mental Health inpatient services HealthPoint page](#) for C-L Psychiatry operating hours and contact details
- Complete all referrals via eReferral
- Referrals will be triaged by the Triage Nurse or Consultant Psychiatrist and an appropriate team member allocated
- If there is an imminent risk to the safety of the patient or others, a Code Black should be activated.

ED and inpatient wards

Behaviour Support Team (BeST)

The **RPH Behaviour Support Team** provides inreach services for expert consultancy in the identification and management of patients with behaviours of concern.

- Requests for support from BeST is via phone or during proactive rounding.

Alcohol and Other Drug C-L Service Support

- **RPHBG Specialist input and support covering ED and inpatients**
 - **08:00 – 16:00 hours, Mon – Sun** contact via Switchboard or on [Alcohol and Other Drugs Services](#) SharePoint page
 - **Statewide Drug and Alcohol Clinical Advisory Service (DACAS 08:00 – 20:00 hours – 08 6553 0520).**

Appendix I: Specialist Referral Pathways – General Health cont'd

Inpatient services – Bentley Health Service (BHS) General

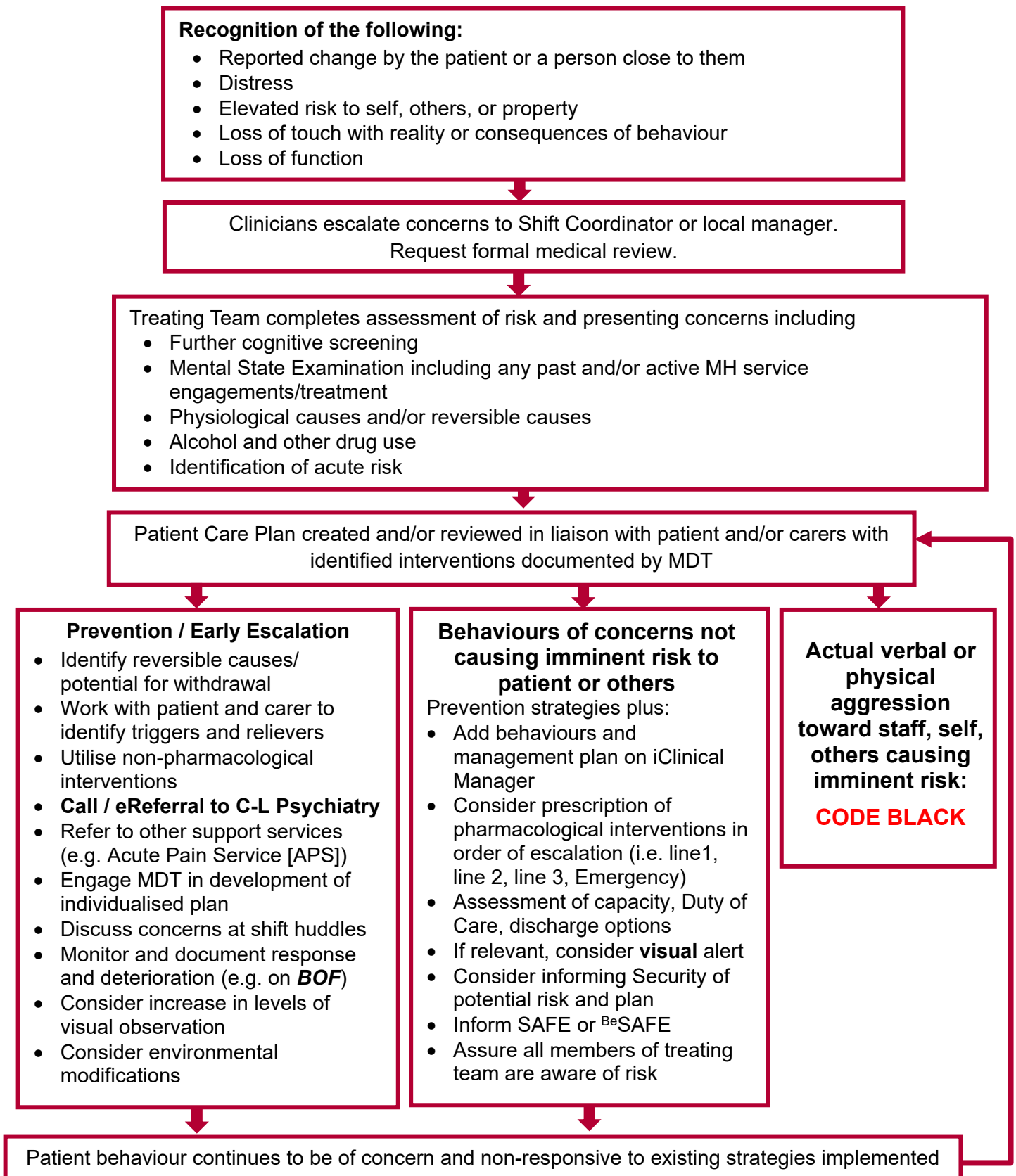
In hours (Mon-Fri 08:00-17:00 hours)

- **Non-urgent** eReferral to be completed for C-L input and review. All patient referrals MUST be via eReferral
- C-L can be contacted by phone for advice
- **If urgent** or C-L unavailable, home team medical officer to contact allocated Duty Medical Officer (DMO) via Switchboard
- The DMO will discuss the case and arrange patient review. They can also discuss and escalate to the MH Consultant.

Out of Hours (Monday to Friday 17:00 – 08:00 hours and Saturday, Sunday, and Public Holidays)

- All concerns regarding altered mental state are to be escalated to BeSAFE as per initial escalation protocol
- Following review, if urgent MH review is required, BeSAFE to contact BHS After Hours MH CNS who will liaise with On Call MH DMO for initial review
- Referring and responding clinician(s) to agree on time for review and any interim actions (including medications, type and frequency of observations, requirement for special or security)
- DMO will assess and liaise with Psychiatrist on call as required
- Plan will be documented in the DMR.

Appendix II: RRAD Mental State Escalation Pathway – RPBG General Health Inpatient Service



Appendix III: MH Division Assessment Tools

Assessment and Information-gathering tools – Inpatient services

The following clinical documents are available for use as clinically indicated during the patient's care, e.g., admission, at review and when changes in mental state and/or behaviour occur, and provide an overall clinical picture of the patient's mental state, including:

- *Mental Health Assessment Form (MHAF)*
- *RAMP*
- *Physical Examination Form, Physical Appearance and Adult Observation and Response Chart (AORC)* where relevant
- *Mental Health Transcultural Assessment Form* for patients identified as being from a Culturally and Linguistically Diverse (CaLD) background
- *MCCP, What Matters Most Record (WMMR) and Treatment Support and Discharge Plan (TSDP)*
- Age-appropriate National Outcomes Case-Mix Collection (NOCCS), e.g.
 - Health of the Nation Outcome Scales (HoNOS)
 - MH Phase of Care (MH POC)
 - *LSP 16*
 - *Kessler 10+LM* offered on admission
 - *Kessler 10L3D* offered at review and discharge
- **Community Mental Health Services Triage to Discharge SOP**
- **WA Department of Health Department Statewide Standardised Clinical Documentation (SSCD)**
- Drug and Alcohol Screening Tools (*AUDIT and DAST-10*) and Nicotine Screening Tools
- *Montreal Cognitive Assessment (MoCA)* or *Mini Mental State Examination (MMSE)* and Rowland Universal Dementia Assessment Scale (RUDAS) for older adult patients

Assessment and monitoring tools – Community Services

- AORC (adults only) can be completed by Care Coordinator or Wellness Staff – (available in some community services i.e., Eating Disorder Specialist Service)
- SSCD – *MHAF, RAMP, Physical Appearance Form*
- Crisis Awareness Plan, *TSDP*
- *Clozapine Monitoring Form* to provide records of monitoring and side effect assessments
- *Intramuscular Long-Acting Injection (LAI) Chart* to assist clinicians in the safe prescription, administration and monitoring in community settings
- Age-appropriate NOCCS, e.g.
 - HoNOS
 - MH Phase of Care (MH POC)
 - *LSP 16*
 - *Kessler 10+LM* offered on admission
 - *Kessler 10L3D* offered at review and discharge
- *Alcohol, Smoking and Substance Involvement Screening Tool (ASSIST-Lite)*.

For further tools on measuring specific aspects of a patient's mental state, consider state-wide assessment tools and resources are available from [WA Department of Health Mental Health Unit website](#).

Appendix IV: RRAD Mental State Escalation Pathway – MH Division Inpatient Services

Identification of deterioration in Mental State Recognition of the following:

- Reported change by the patient or a person close to them
- Distress
- Elevated risk to self, others, or property
- Loss of touch with reality or consequences of behaviour
- Loss of function

Initial Interventions

- Clinician who is recipient of information to provide initial response (including asking “what can we do to help?” and offering appropriate support).
- Clinician provides assessment of behaviour, thinking and risk issues ensuring safety and assessing level of risk to patient, self, others and/or environment including risk of vulnerability to sexual assault
 - Sexual safety risks/sexually inappropriate behaviour are a trigger for requiring escalation of care
- Clinician provides immediate support and utilise de-escalation strategies to reduce distress
- Collaborate with the patient on appropriate strategies (e.g., WMMR, preferred treatment options, communicate with family and/or PSPs). For TCU only, include MINDS My Better Life Plan, Client Wellness Plan
- Increase frequency of visual observation if appropriate
- Consider environmental modifications on the ward or service such as reduced stimulation, access to comfort room and/or removal of articles that could be used for harm or other. Refer to **Prevention and Management of Aggressive Behaviours Within MH Inpatient SOP**
- Offer PRN as required and prescribed

Escalation and Referral

- If safe to do so, report deterioration to Shift Coordinator, Allocated Nurse, MO, DMO, Treating Team, treating Consultant or Duty Consultant (on call) to continue interventions
- Consider urgent medical review, medication review, report to allocated clinician and/or treating team for further assessment and interventions
- Inform and discuss with appropriate MDT
- Referral to Allied Health for specified assessment and interventions where appropriate.
- Complete other appropriate referrals
- Consider liaison with SAFE or BeSAFE team to identify patient of concern
- Review for **MHA 2014** applications and requirements

Emergency

- If urgent assistance is required, activate appropriate emergency response (e.g., Code Black, Code Blue)

Post Incident and Documentation

- Document all interventions and outcomes in patient's integrated notes and in webPSOLIS
- Ensure changes are reported during clinical handover and safety huddles for appropriate review
- Update Risk Assessment documentation including *RAMP*, *Progressive Risk Assessment (PRA)* PSOLIS *TSDP*, and iClinical Manager
- Check and update alerts in PSOLIS
- Update *WMMR* and individualised *Patient Care Plan*
- Complete a NOCC review and MH Phase of Care
- Liaise with SAFE or BeSAFE re: possible inclusion on Patients of Concern (PoC) list

Post Emergency

- Ensure patients and staff are offered opportunity for debrief and/or additional support Refer to **Mental Health Patient, Carer and Visitor Support Post-Critical Incident SOP** and **EMHS Post Incident Management/Critical Incident Stress Debriefing Policy**

Appendix V: RRAD Mental State Escalation Pathway – MH Division Community Services

Identification of deterioration in Mental State Recognition of the following:

- Reported change by the patient or a person close to them
- Distress
- Elevated risk to self, others, or property
- Loss of touch with reality or consequences of behaviour
- Loss of function

Initial Interventions

Initial Contact with Unknown Patient

- Clinician who is recipient of information to provide initial response (including asking “what can we do to help?” and offering appropriate support).
- Request for Triage via Assessment Treatment Team (ATT)
- Clinician provides immediate support and collaborates with the patient on appropriate strategies
- **Escalate** frequency of home visits if applicable (CMHS)

Activated Consumer

- Create or review safety plan with strategies for both patient and family and/or PSP to seek assistance and support if risks continue to escalate after hours, (e.g., contacting MH Emergency Response Line [MHERL], when to attend ED, Telehealth consultation)
- Discuss with patient and PSPs safe use of PRN medication, how much (dose) and when to take it

Escalation and Referral

- Liaise concerns immediately with team consultant and review/amend current interventions
- Discuss case with MDT at next MDT meeting – review and strengthen care strategies
- Referral to Allied Health for specified assessment and interventions where appropriate.
- Complete other appropriate referrals, e.g. Safe Haven/external community services
- Review for **MHA 2014** applications and requirements

Emergency

- Liaise with Authorised MH Practitioner (AMHP) for *MMSE*, capacity and risks assessment, Forms 1A completion and form 4A if PSP or other means are unavailable to safely transport patient to ED
- Contact WAPOL to advise of identified risk(s) and request welfare check obtain job number and recommendation(s) and document all details (advised and received).

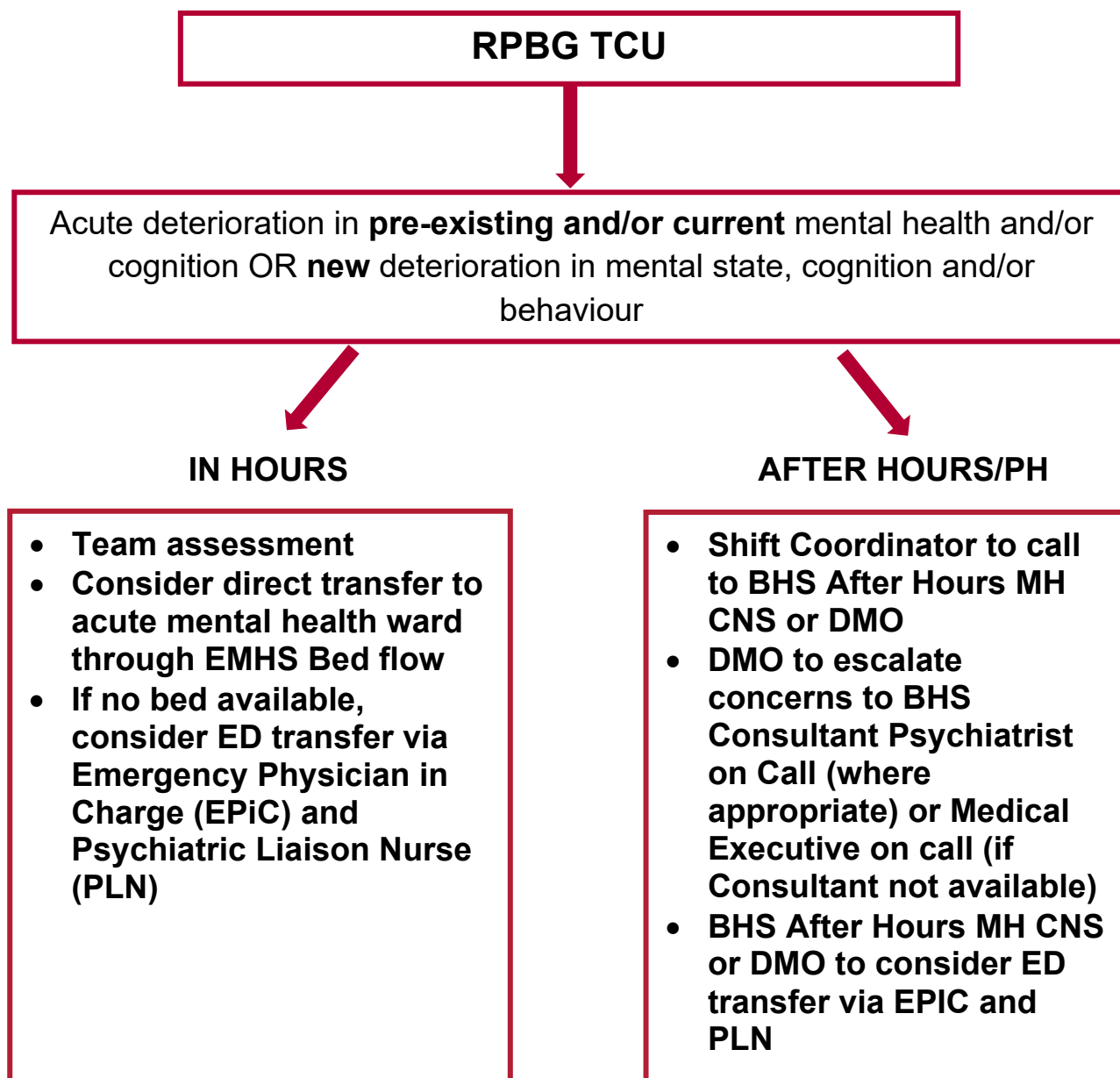
Documentation

- Document all interventions and outcomes
- Complete a NOCC review and MH POC to capture change in clinical presentation as soon as practicable
- Review and complete a new *RAMP* as soon as practicable
- Review and complete changes to *TSDP* and Crisis Awareness Plan as soon as practicable
- If consumer has been admitted to inpatient setting liaise with ward staff and complete verbal iSoBAR handover – request to be informed of discharge proceedings and a phone call on day of discharge
- Follow up care within 7 days (in PSOLIS Service event)
- Check and update Alerts in PSOLIS

Post Incident and Emergency

- Follow up with family, PSPs refer and/or offer carers support services (as per **Mental Health Patient, Carer and Visitor Support Post-Critical Incident SOP**)
- For staff debrief, refer to **EMHS Post Incident Management/Critical Incident Stress Debriefing Policy**
- Attend a debrief with Clinical supervisor and/or formal debrief offered by Service
- Contact Employee Assistance Programme (EAP) if required

Appendix VI: Acute Deterioration in Mental State at Bidi Wungen Kaat Centre (TCU)



Refer to **EMHS Mental Health Patient Flow and Bed Capacity Framework**

Appendix VII: Acute Deterioration in Mental State at Next Step Inpatient Withdrawal Unit (IPWU), Outpatients and Integrated Services Escalation Pathway

Next Step IPWU, Outpatients East Perth and Integrated Services

Acute deterioration in **pre-existing and/or current** mental health and/or cognition OR **new** deterioration in mental state, cognition and/or behaviour

Call 000 for an ambulance in all emergency situations at any site.

Call 000 for police welfare check if patient leaves prior to medical assessment or ambulance arriving.

Staff to contact WA Police (WAPOL) to advise of identified risks and request a welfare check. Clinician to obtain job number and WAPOL recommendations and document all details advised and received.

If Dr on site

- Shift Coordinator / Clinical Nurse Manager / Clinical Manager to escalate MO to request immediate review
- MO to consider direct transfer to ED via Emergency Physician in Charge (EPiC) and Psychiatric Liaison Nurse (PLN) or form consumer under Mental Health Act 2014 (Form 1A) if not voluntarily willing to go to ED for assessment
- NUM / CoN / Program Manager to be advised.

If Dr not on site

Integrated Services

- Clinical Nurse Manager / Clinical Manager / Clinical Nurse to call 000 for ambulance

IPWU

- IPWU Shift Coordinator to escalate to On Call MO to request assessment and advice.
- On call MO to consider the need for direct transfer to ED via Emergency Physician in Charge (EPiC) and Psychiatric Liaison Nurse (PLN)
- Operations Hub CNM to be advised.

Appendix VIII: RRAD Mental State Escalation Pathway – RPBG General Health Outpatient Services

